

Surgery for cervical cancer

Surgery is the main treatment for stage 1 cervical cancer. It is also sometimes used to treat small stage 2A cancers.

On this page

- [Types of surgery for remove cervical cancer](#)
- [Large loop excision of the transformation zone \(LLETZ\)](#)
- [Cone biopsy](#)
- [Hysterectomy](#)
- [Trachelectomy](#)
- [How the operation is done](#)
- [About our information](#)
- [How we can help](#)

Types of surgery for remove cervical cancer

Surgery is the main treatment for stage 1 [cervical cancer](#). It is also sometimes used to treat small stage 2A cancers.

There are different types of surgery to remove cervical cancer. These are:

- large loop excision of transformation zone (LLETZ)
- cone biopsy
- hysterectomy
- trachelectomy.

The type of operation you have will depend on several things, including:

- the [stage of the cancer](#)
- the size of the cancer
- whether you have been through the menopause
- whether you want to get pregnant in the future.

Whatever type of operation you have, the aim is to remove all of the cancer. The surgeon will remove the cancer and a margin of healthy tissue around it. Depending on the type of operation you have, they may also remove other tissue. After the operation, the surgeon will send all the tissue to a laboratory to be looked at under a microscope.

You may need further treatment to reduce the risk of the cancer coming back if cancer cells are found in:

- tiny blood vessels or lymph vessels inside the tumour
- [lymph nodes](#)
- tissue around the cervix.

Your surgeon will usually discuss these results with you at your first follow-up appointment after the operation.

Large loop excision of the transformation zone (LLETZ)

You may have had a [large loop excision of transformation zone \(LLETZ\)](#) as part of your diagnosis of cervical cancer. It is sometimes called loop electrosurgical excision procedure (LEEP). This operation is often used to treat the earliest stage of cervical cancer (stage 1A1). It may be the only treatment you need.

Cone biopsy

A [cone biopsy](#) is a small operation to remove an area of the cervix. This operation may be used to treat stage 1 cervical cancer.

Hysterectomy

A [hysterectomy](#) is an operation to remove the womb. It is the standard treatment for early-stage cervical cancer. If you have been through the menopause, the surgeon will usually also remove the fallopian tubes and ovaries. Sometimes the surgeon needs to remove the ovaries if you have not had the menopause. This means your periods will stop straight away and you will have the [menopause](#).

After a hysterectomy, you will not be able to get pregnant. Being told that your cancer treatment means you cannot get pregnant can be very difficult.

If you are told you need to have a hysterectomy, you can ask your hospital doctor to refer you to a fertility specialist before your surgery. They can explain [fertility options](#) to you. If you are interested in surrogacy (someone else carrying a child in their womb for you), you may want to store eggs or embryos (fertilised eggs).

There are two types of hysterectomy. The type you have depends on the stage of the cancer.

Types of hysterectomy

Simple hysterectomy (also called total hysterectomy)



The surgeon removes the womb and cervix. They may also remove your pelvic lymph nodes. This operation may be done for very early-stage cancers that cannot be seen without a microscope (stage 1A).

Radical hysterectomy



The surgeon removes the upper part of the vagina, the cervix, the womb, the pelvic lymph nodes and the tissue surrounding the womb and cervix (parametrium).

Trachelectomy

This operation is a type of fertility-sparing surgery. It may be an option if you want to get pregnant in the future and have early-stage cervical cancer.

The surgeon removes the cervix and the upper part of the vagina. They usually also remove the supporting tissues around the cervix. This is called a radical trachelectomy.

[Pelvic lymph nodes](#) are also removed. The surgeon usually does this through small cuts in your abdomen. This is called laparoscopic surgery. It may be done a few days before, or at the same time as, the trachelectomy.

The womb is left in place so that it is possible for you to get pregnant in the future. The surgeon usually puts a stitch at the bottom of the womb after removing the cervix. This helps to keep the womb closed during pregnancy. After a trachelectomy, there is a higher chance of miscarrying during pregnancy. If you become pregnant, you will be referred to a local specialist maternity service for closer monitoring. The baby will need to be delivered by caesarean section. Your surgeon can explain more about this.

A trachelectomy is very specialised surgery and is not done in all cancer hospitals. If it is an option for you, you may need to be referred to another hospital. There, you can discuss the benefits and possible risks with a surgeon who specialises in this operation.

How the operation is done

A hysterectomy or trachelectomy can be done in different ways.

Abdominal surgery



The surgeon makes one large cut (incision) in the tummy (abdomen). Afterwards, you have a wound that goes across your tummy just above your hips, or that goes up your tummy from just above your hips.

Vaginal surgery



The surgeon operates through a cut at the top of the vagina. The surgeon may combine this with laparoscopic or robotic surgery.

Laparoscopic surgery



The surgeon operates through small cuts in the tummy. They use small surgical instruments and a thin telescope with a video camera on the end (laparoscope). The laparoscope lets the surgeon see inside the body.

Robotic surgery



This is like laparoscopic surgery, but the laparoscope and instruments are attached to robotic arms. The surgeon controls the robotic arms, which can move very delicately, steadily and precisely.

Your surgeon will talk with you about the type of surgery you will have.

About our information

References



Below is a sample of the sources used in our cervical cancer, cervical screening and CIN information. If you would like more information about the sources we use, please contact us at canцерinformationteam@macmillan.org.uk

GOV.UK. Cervical screening: programme overview. Updated 18 November 2019. Available from www.gov.uk/guidance/cervical-screening-programme-overview (accessed March 2020).

GOV.UK. Colposcopic diagnosis, treatment and follow up. Updated 5 February 2020. Available from www.gov.uk/government/publications/cervical-screening-programme-and-colposcopy-management/3-colposcopic-diagnosis-treatment-and-follow-up (accessed April 2020).

Marth C, Landoni F, Mahner S, et al. Cervical cancer: ESMO clinical practice guidelines. Annals of Oncology, 2017; 28, suppl 4, iv72–iv83. Available from <https://www.esmo.org/guidelines/esmo-clinical-practice-guideline-cervical-cancer> (accessed October 2020).

Reed N, Balega J, Barwick T, et al. British Gynaecological Cancer Society (BGCS) cervical cancer guidelines: recommendations for practice. 2020. Available from www.bgcs.org.uk/wp-content/uploads/2020/05/FINAL-Cx-Ca-Version-for-submission.pdf (accessed October 2020).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Professor Nick Reed](#), Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 April 2021 | Next review: 01 April 2024

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information.](#)

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help